



The wearable clinical co-pilot that carries context forward.

Carry listens during the visit, privacy-filters every word on device, drafts the note, and remembers what matters across visits, so the next conversation starts where the last one ended.

Doctor Mode

First profession pack

Privacy-first

De-identified before processing

Review-first

Every clinical output is a draft

A doctor who spends the evening finishing notes from the morning.



Dr. Rao, primary care physician

Sees 25 to 35 patients a day. Returning patients, new patients, overlapping histories.

1 During the visit

- Splits attention between the patient and the keyboard.
- Tries to recall what changed since the last visit.
- Holds allergies and medications in memory.

2 After the visit

- Writes the SOAP note from scattered memory.
- Sets coding, follow-ups, and referrals by hand.
- Documentation spills into personal time.

3 Across visits

- Prior context lives in long charts no one re-reads.
- A known allergy can be missed under time pressure.
- Continuity depends on one person remembering.

Documentation steals time, and continuity is fragile.

Two costs sit on every clinician. The administrative load that follows each visit, and the risk that critical context from earlier visits does not surface at the moment it matters.

Time lost to administration

- Clinicians spend close to **two hours on paperwork for every hour** of patient care.
- Notes are reconstructed from memory, hours after the conversation.
- Coding, follow-ups, and referrals are manual and easy to defer.

Context that does not carry

- A **penicillin allergy** noted last visit may not surface before the next prescription.
- Prior medications and decisions are buried in long charts.
- Continuity depends on a human re-reading everything, every time.

The same visit, with Carry listening.

BEFORE

The clinician opens the room with no briefing. They recall the last visit from memory.

PRE-READ

Carry presents a one-screen briefing built from earlier visits: known allergies, active medications, and open questions, carried forward automatically.

DURING

Carry listens, de-identifies on device, and **tracks clinical changes live**: symptoms, medication decisions, safety conflicts.

END

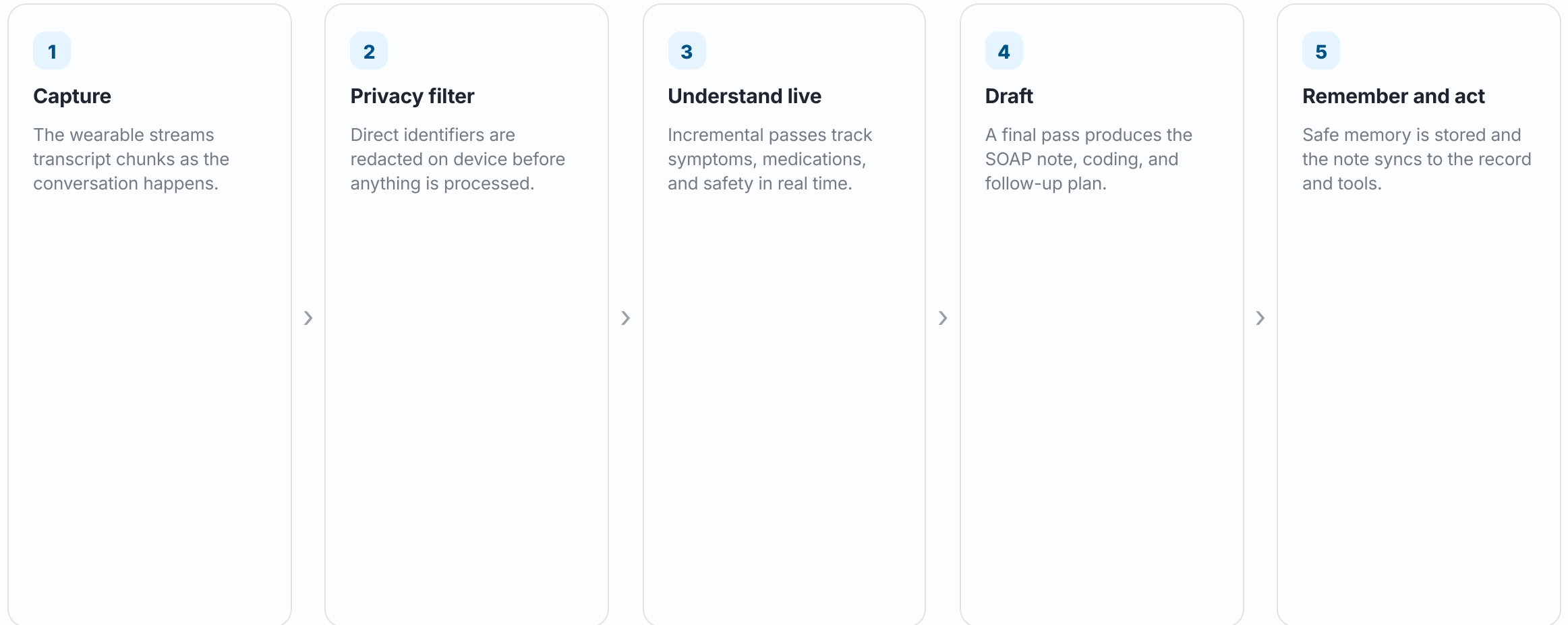
A **SOAP note, coding suggestions, and a follow-up plan** are drafted in seconds, each clearly marked for clinician review.

AFTER

The note syncs to the record, the follow-up is prepared, and **what was learned is carried into the next visit.**

• HOW IT WORKS

One pipeline, from spoken words to reviewed work.



Streaming transcript

On-device de-identification

Live clinical tracking

SOAP and ICD-10 drafts

Longitudinal memory

Notion and calendar sync

Today: the pre-visit briefing

A returning patient. The briefing is built only from what Carry captured in earlier visits, with known allergies and medications carried forward.

[Live dashboard](#)

C

Carry

Today
Command center

Live Visit
Active conversation

Patient
Longitudinal record

Timeline
Clinical history

Knowledge Graph
Relationships

TUESDAY CLINIC

Good morning, Dr. Rao

DEMO Visit 1 Visit 2 Reset record [Begin next visit](#)

AM Anaya Mehta
34, she/her · MRN-4821 · 09:30
Returning patient

Pre-visit briefing, built from what Carry captured in earlier visits.

CARRIED FORWARD BY CARRY
Penicillin allergy Cetirizine 10 mg PO PRN

LAST VISIT
Started Cetirizine 10 mg PO PRN

CURRENT MEDICATIONS
Cetirizine 10 mg PO PRN

ALLERGIES
Penicillin

RECENT SYMPTOMS
Sneezing Nasal congestion Itchy eyes

UNRESOLVED
• Return if: fever, sore throat, unimproved congestion
• Confirm symptom course since last visit

SUGGESTED AGENDA
1 Characterize current sore throat and fever
2 Confirm known allergies before any antibiotic
3 Assess need for rapid strep testing

Needs attention

1 Notes captured Visit drafts on record

1 Allergies known Carried into every visit

1 Active medications From captured visits

0 Conditions tracked Across the record

Recent updates

• Knowledge graph and timeline now span both visits

• Record updated from the latest visit

• Symptom noted: congestion

• Symptom noted: stuffy nose

• Allergy confirmed

 Carry

06

Live Visit: the safety catch

An antibiotic is proposed, the patient states a penicillin allergy, and Carry strikes the unsafe drug, flags the conflict, and tracks the safe alternative, as it happens.

Allergy conflict detected

C

Carry

Today
Command center

Live Visit
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Relationships

LIVE VISIT · ANAYA MEHTA · VISIT 2

Conversation

4 exchanges

Clinician

Welcome back. What brings you in today?

Patient

CAPTURED

My name is Anaya Mehta. I have had a sore throat, fever, and swollen glands for four days.

SENT FOR PROCESSING

My name is [PRIVATE_PERSON_1]. I have had a sore throat, fever, and swollen glands for four days.

1 item redacted before anything leaves the device

Clinician

Any cough, chest pain, or difficulty breathing?

Patient

No cough, no chest pain, and breathing is okay.

Clinical intelligence

Updated live

NOTE DRAFT

Drafting

S

Sore throat, fever, and swollen glands, 4 days

O

sore throat, fever, swollen glands

A

Working assessment in progress

P

SYMPTOMS

sore throat

fever

swollen glands

MEDICATIONS

None mentioned yet

SAFETY CHECKS

Drafted actions

Calendar, lab, and sync drafts appear here as the visit progresses.

Final draft, ready for review

SOAP note, medication decision trail, coding suggestions, and a follow-up plan. Every item is a draft that requires clinician sign-off before anything is final.

Clinician review required

C

Carry

Today

Command center

Live Visit

Active conversation

Patient

Longitudinal record

Timeline

Clinical history

Knowledge Graph

Relationships

LIVE VISIT · ANAYA MEHTA · VISIT 2

Conversation

Live transcript

10 exchanges

Patient

This could be bacterial pharyngitis. I was going to start amoxicillin 500 milligrams by mouth twice daily for ten days.

Clinician

Wait, I am allergic to penicillin and amoxicillin. I get hives and swelling.

Clinician

Thanks for confirming. Do not take amoxicillin. We will avoid penicillin medicines.

Clinician

Instead, I will use azithromycin: 500 milligrams by mouth today, then 250 milligrams by mouth once daily for the next four days.

Patient

Okay. Should I watch for anything?

Clinical intelligence

Updated live

NOTE DRAFT

Ready for review

S

Sore throat, fever, and swollen glands for 4 days. The patient presents with a 4-day history of sore throat, fever, and swollen glands. Symptoms are acute. Patient denies cough, chest pain, or difficulty breathing. Allergic to penicillin and amoxicillin, presenting with hives and facial swelling. None currently reported active for this acute episode, though patient has a history of cetirizine use for seasonal allergic rhinitis.

O

Not recorded in transcript. Not performed/discussed in transcript.

A

Acute bacterial pharyngitis true

P

None ordered during this encounter. Initiate Azithromycin 500 mg PO on Day 1, followed by 250 mg PO daily on Days 2-5. Amoxicillin was initially proposed but cancelled due to a confirmed severe penicillin allergy. Instructed to seek urgent care if experiencing trouble

Drafted actions

Visit complete

Begin next visit

Notion synced

Draft

Visit note created.
<https://app.notion.com/p/Carry-Doctor-Visit-Draft-2026-06-20-45247467-385ed79b50018148b401e0c9457a7d4d>

Notion sync

Draft

Preparing visit note for sync.

Calendar draft

Draft

Follow-up: two days.

Medication update

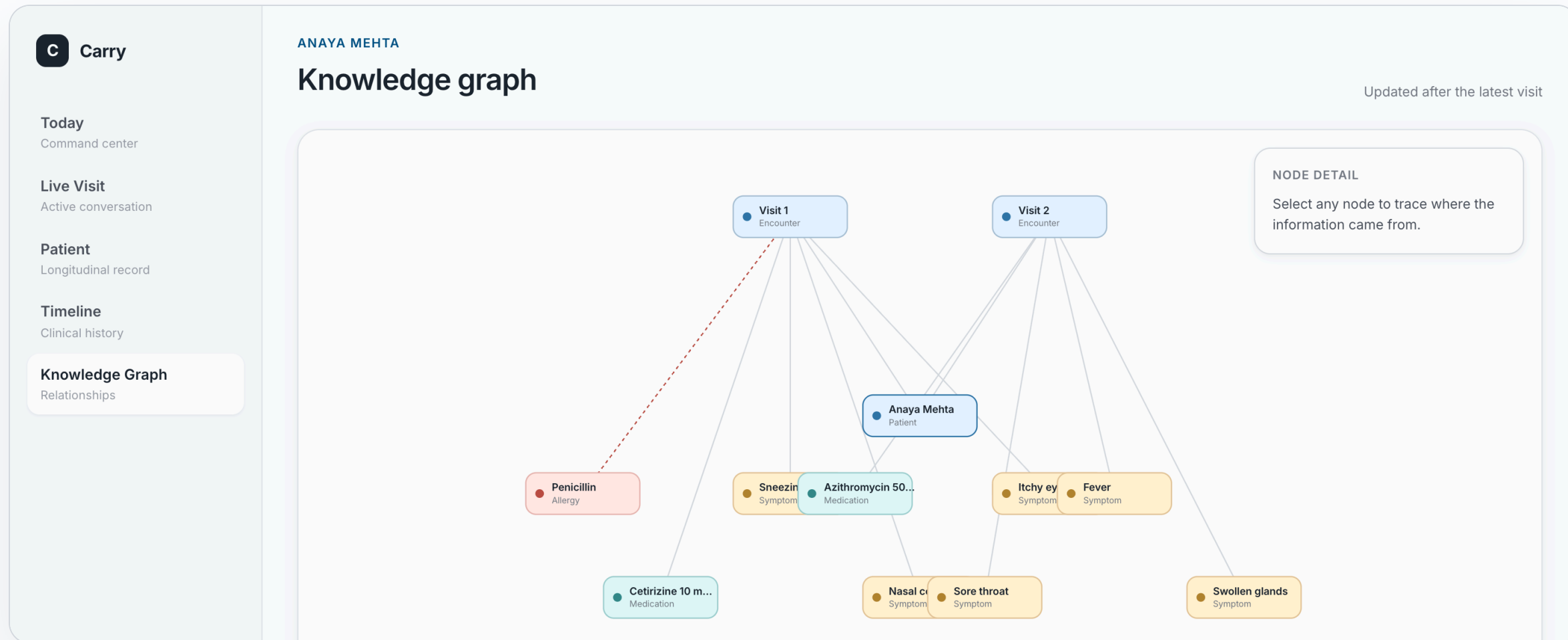
Draft

azithromycin 500 milligrams by mouth today, then 250 milligrams by mouth once daily for the next four days added as active draft.

Continuity: the patient knowledge graph

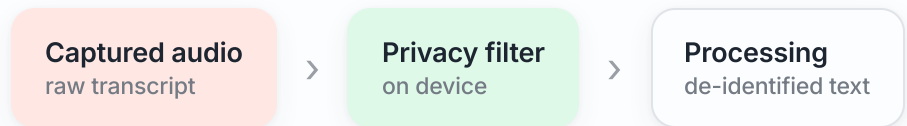
Every captured fact is wired to the visit where Carry learned it. The graph and timeline are built from real stored memory, not a fixed layout, and grow with each visit.

Built from live memory



- PRIVACY

De-identified before anything leaves the device.



- Privacy filtering is the **first step** after capture, before any model sees the text.
- Direct identifiers are masked. **Clinical durations and relative dates are preserved** so the note stays accurate.
- Stored memory is filtered too: a blocklist keeps sensitive fields out of long-term context.
- Privacy-first, compliance-oriented architecture. The proof is visible in the transcript.

- CAPTURED

My name is **Anaya-Mehta**. I have had a sore throat and fever for four days.

- SENT FOR PROCESSING

My name is **[PRIVATE_PERSON_1]**. I have had a sore throat and fever for four days.

The dashboard shows both blocks side by side during the live visit, so the de-identification is auditable, not assumed.

Runs where the data has to stay.

Carry is built as a generic core with profession packs. Nothing in the architecture requires a public cloud, which makes on-premise and private deployment a configuration choice, not a rewrite.

On-premise	The full pipeline runs inside the clinic network. Audio, transcripts, and memory never leave the building.
Local storage	Patient memory is held in a local database. Reset, export, and retention are controlled by the clinic.
Pluggable models	The language model is reached through a standard interface, so a self-hosted model can replace the hosted one with a config change.
Config not code	Secrets live in environment variables, behavior lives in YAML. Privacy rules and tool access are set per deployment.
Optional integrations	External sync to records and calendars is opt-in and scoped , and can be turned off entirely for air-gapped sites.

Generic core, profession packs on top.

C Core platform

- Streaming transcript ingestion
- Privacy filter service
- Incremental and final passes
- Memory store and audit log

D Doctor pack

- SOAP note drafting
- Medication decision tracking
- Safety and allergy checks
- ICD-10 and follow-up plans

A Actions

- Record sync to Notion
- Calendar follow-ups
- Scoped, opt-in connections
- Every action is accountable

The next profession pack reuses the entire core. Only the outputs, prompts, and memory schema change.

- CARRY

Carry the conversation into the work, and into the next visit.

SAVES TIME

The note, the coding, and the follow-up are drafted before the clinician leaves the room.

SAFER CONTINUITY

What was learned is carried forward, so a known allergy surfaces at the right moment.

DEPLOYS PRIVATELY

Privacy-first by construction, and able to run entirely inside the clinic.